This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction
for the administration of Salbutamol 100mcg MDI inhaler and Prednisolone 5mg tablets for the treatment of Asthma Rescue

Summary of NICE / NICE CKS Guidelines for Asthma Rescue

Asthma Overview

- **Definition:** Chronic inflammatory condition of the airways presenting with recurrent episodes of wheezing, breathlessness, chest tightness, and cough.
- **Diagnosis:** Based on clinical history and pulmonary function testing (spirometry); must be confirmed prior to this PGD.

Acute Exacerbation Assessment

- **Features of acute exacerbation:** Worsening breathlessness, wheezing, cough, chest tightness not relieved by usual reliever therapy.
- **Severity assessment:** Mild-moderate exacerbation (able to complete sentences), versus severe (unable to speak full sentences, distress, low oxygen saturation).
- **Red flags:** Silent chest, exhaustion, confusion, or hypoxia warrant emergency referral.

Rescue Treatment

- **Short-acting beta-2 agonist (SABA):** First-line for acute symptom relief (salbutamol).
- **Oral corticosteroids:** For acute exacerbations to reduce airway inflammation (prednisolone).
- **Response monitoring:** Assess response at 15-30 minutes; if insufficient relief, refer for emergency assessment.

Patient Group Direction

for the supply/administration of

Salbutamol 100mcg MDI inhaler

for the treatment of

Asthma Rescue

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training

	relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must previously have used PGDs to supply medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Acute symptom relief in adults with diagnosed asthma presenting with acute exacerbation (breathlessness, wheezing, chest tightness)
Inclusion criteria	Adults aged 18 years and over Confirmed diagnosis of asthma (documented in GP records or previous inhaler use) Acute exacerbation with symptoms of bronchospasm (wheezing, breathlessness, chest tightness) Capable of using inhaler device or willing to use spacer Capable of giving valid informed consent
Exclusion criteria	Known hypersensitivity to salbutamol or other beta-2 agonists Severe exacerbation with hypoxia (SpO2 <92%) - refer for emergency assessment Silent chest, exhaustion, confusion - refer for emergency assessment First presentation suggestive of asthma without prior diagnosis - refer for assessment
Cautions	Cardiovascular disease: beta-2 agonists can increase heart rate and blood pressure Diabetes mellitus: monitor blood glucose (hyperglycaemia can occur) Hyperthyroidism: beta-2 agonists can worsen symptoms Hypokalaemia: may be worsened by beta-2 agonists; monitor potassium levels Pregnancy: salbutamol is safe but ensure appropriate informed consent
Actions if patient is excluded or	Advise on alternative treatment options and how these can be

declines treatment	accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Salbutamol 100mcg metered-dose inhaler (MDI)
Legal category	POM
Storage	Store below 25°C. Keep away from heat and direct sunlight. Shake well before each use.
Route/method of administration	Inhalation via MDI, with or without spacer
Dose and frequency	2-4 puffs as required for acute symptom relief Initial dose: 2-4 puffs inhaled immediately for acute exacerbation Use spacer device in less experienced patients to optimise drug delivery May repeat after 15-30 minutes if inadequate response
Quantity to be administered and/or supplied	1 MDI inhaler (200 doses)
Maximum or minimum treatment period	As needed during acute exacerbation
Adverse effects	Common: tremor (fine tremor of hands), headache, nervousness, tachycardia Uncommon: palpitations, muscle cramps, anxiety Rare: hypersensitivity reactions, arrhythmias, myocardial ischaemia

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • date of supply dose, form and route quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear,

	legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with each medication.
Follow-up advice to be given to patient or carer	Use the inhaler technique sheet provided to ensure optimal drug delivery. If using a MDI without a spacer, coordinate inhalation with actuation. If symptoms do not improve within 15-30 minutes of salbutamol use, seek emergency medical attention. Take the full course of prednisolone as prescribed, even if symptoms improve. Take prednisolone with food if it causes stomach upset. Do not stop prednisolone abruptly; follow the course as directed. If diabetic, monitor blood glucose more frequently as prednisolone may raise levels; inform your GP. If taking other medications, inform your healthcare provider of steroid use. Seek immediate medical attention if experiencing severe breathlessness, chest pain, confusion, or exhaustion during the exacerbation. Review your asthma action plan and triggers with your GP after recovery. Ensure your asthma maintenance therapy is optimised to prevent future exacerbations.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 https://www.nice.org.uk/guidance/mg2
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 https://www.nice.org.uk/guidance/mpg2/resources
- NICE CKS Guidance: Asthma: diagnosis and management
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date	Expiry date		
1/11/25	31/10/26		

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		2/11/25
Chris Pilkington	Pharmacist GPhC: 2046322		2/11/25

Change history

Version number	Change details	Date
001	Development & issue of new PGD	1st November 2025

Patient Group Direction

for the supply/administration of

Prednisolone 5mg tablets

for the treatment of

Asthma Rescue

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must previously have used PGDs to supply medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Treatment of acute asthma exacerbation to reduce airway inflammation and improve bronchospasm
Inclusion criteria	Adults aged 18 years and over Confirmed diagnosis of asthma Acute exacerbation inadequately responsive to SABA alone Able to take oral medication No contraindications to corticosteroid use Capable of giving valid informed consent
Exclusion criteria	Known hypersensitivity to prednisolone or other corticosteroids Systemic infection (unless treated with appropriate antimicrobials)

	Vaccination with live vaccines during treatment Severe hepatic dysfunction Uncontrolled hypertension or cardiac disease (relative - discuss risk/benefit)
Cautions	Diabetes mellitus: corticosteroids can increase blood glucose; monitor BM and adjust diabetes medication if needed Osteoporosis or risk factors: short-term treatment risk is low but inform patient Peptic ulcer disease or GI upset: consider gastroprotection Psychiatric history: corticosteroids can cause mood changes, insomnia, or exacerbate existing conditions Renal impairment: generally safe for short-term use Hypertension: may worsen; monitor blood pressure Infections: can mask symptoms; ensure appropriate investigation before use Pregnancy and breastfeeding: generally safe for acute use; discuss benefits/risks
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Prednisolone 5mg tablets
Legal category	POM
Storage	Store below 25°C. Keep in original container. Protect from light and moisture.
Route/method of administration	Oral
Dose and frequency	40-50mg as a single dose or in divided doses for acute exacerbation Typical: 40mg daily for 5-7 days for acute exacerbation May be taken with food to reduce GI upset For 5-7 day course: 8-10 tablets of 5mg strength
Quantity to be administered and/or supplied	Up to 42 tablets (5mg) for a single course
Maximum or minimum treatment period	5-7 days for acute exacerbation
Adverse effects	Very common: insomnia, mood changes (euphoria, irritability,

	anxiety), increased appetite Common: GI upset (dyspepsia, nausea), increased sweating, headache, dizziness Uncommon: acne, facial flushing, restlessness, hypertension, hyperglycaemia, immunosuppression Rare: adrenal suppression (with prolonged use), osteoporosis, avascular necrosis, psychiatric disturbances, opportunistic infections

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • date of supply dose, form and route quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

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	If diabetic, monitor blood glucose more frequently as prednisolone may raise levels; inform your GP. If taking other medications, inform your healthcare provider of steroid use. Seek immediate medical attention if experiencing severe breathlessness, chest pain, confusion, or exhaustion during the exacerbation. Review your asthma action plan and triggers with your GP after recovery. Ensure your asthma maintenance therapy is optimised to prevent future exacerbations.

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